



1 PATIENT DEMOGRAPHICS

PATIENT NAME (LAST, FIRST)

DATE OF BIRTH (YYYY / MM / DD)

HEALTH CARD # + VERSION CODE

SEX

M F Other

PATIENT PHONE

SYMPTOM ONSET / DURATION (DATE OR E.G. 8 MONTHS)

ONSET PACE

Rapid Subacute Insidious

days-wks / wks-mos / mos-yrs

2 REFERRING PROVIDER

NAME / CREDENTIALS

CPSO / NP REG #

CLINIC / OFFICE

FAX

PHONE

REFERRING RHEUMATOLOGIST (SHARED CARE, IF ANY)

CONTACT (FAX / PHONE)

3 SUSPECTED PRESENTATION & CLINICAL FEATURES (CHECK ALL THAT APPLY)

SUSPECTED SUBTYPE

Dermatomyositis Necrotizing (IMNM) Antisynthetase / overlap IBM Undifferentiated / possible IIM

Core screen: proximal symmetric weakness with CK elevation. Highest-yield clues are DM skin signs (Gottron, heliotrope) and a positive myositis-antibody panel. A normal CK does not exclude dermatomyositis (amyopathic DM) or IBM. * = high-yield triage clue.

MUSCLE (PATTERN & FUNCTION)

Proximal weakness, symmetric *

Difficulty rising from chair / stairs *

Difficulty lifting arms / combing hair

Rapidly progressive weakness *

Myalgia (muscle pain)

Falls / loss of balance

IBM-specific pattern

Finger-flexor weakness / weak grip *

Quadriceps weakness / knee buckling *

Asymmetric weakness *

Age > 50, slowly progressive *

Little / no response to immunosuppression *

SKIN & NAILS (DERMATOMYOSITIS)

Gottron papules / sign (knuckles) *

Heliotrope rash (periorbital) *

Photodistributed rash (shawl / V-sign)

Mechanic's hands *

Nailfold capillary / periungual change *

Cutaneous ulceration *

Calcinosis

Systemic / antisynthetase

Raynaud's phenomenon

Inflammatory arthritis / arthralgia

Unexplained fever *

Constitutional (weight loss, fatigue)

PULMONARY & BULBAR (RED-FLAG)

Dyspnea / exertional breathlessness *

New / progressive cough

Known / suspected ILD on imaging *

Rapidly progressive dyspnea + hypoxia *

Dysphagia / choking / aspiration *

Dysphonia / nasal regurgitation

Malignancy / mimic screen

New adult-onset DM (cancer screen) *

Recent unexplained weight loss

Statin / myotoxic drug exposure *

Family history of muscle disease

4 INVESTIGATIONS (ATTACH RESULTS)

Strongly recommended: CK, AST / ALT + LDH, ESR & CRP, TSH, CBC, and the myositis-antibody panel (MSA / MAA). Send the antibody panel early: anti-MDA5, anti-Jo-1 / synthetase, anti-SRP and anti-HMGCR change urgency and workup. Do not delay referral awaiting EMG or muscle biopsy.

KEY RESULTS

CK (VALUE / X ULN)		AST / ALT		ANA		NOTABLE ANTIBODY (E.G. ANTI-MDA5)		
				Pos Neg				
MYOSITIS PANEL			EMG			HRCT / ILD		
Sent	Positive	Not done	Irritable myopathy	Normal	Not done	Yes	No	Pending

MUSCLE ENZYMES / LABS

CK *

AST / ALT *

LDH

Aldolase

TSH

CBC

ESR & CRP

MYOSITIS ANTIBODIES (MSA / MAA)

Full myositis panel *

Anti-Jo-1 / synthetase *

Anti-MDA5 *

Anti-TIF1-gamma / NXP2 *

Anti-SRP / HMGCR *

Anti-cN1A (IBM)

ANA

IMAGING / OTHER

EMG / NCS

Muscle MRI (STIR)

HRCT chest (if dyspnea / ILD) *

PFTs incl. DLCO

Muscle biopsy (if done)

Age-appropriate cancer screen (new DM) *

PRIOR TREATMENT TRIED

Corticosteroid DMARD (MTX / AZA / MMF) IVIG Biologic / rituximab

SPECIFY / RESPONSE

CURRENT MEDICATIONS (NOTE STATINS / FIBRATES & OTHER MYOTOXIC AGENTS; LIST OR ATTACH)

REASON FOR REFERRAL

Diagnosis / Management Treatment optimization Second opinion Co-management Juvenile myositis transition

CLINICAL CONCERN (BRIEF SUMMARY)

REFERRING PROVIDER SIGNATURE (TYPE NAME OR SIGN)

DATE (YYYY / MM / DD)

Please attach: relevant lab results, imaging reports, and consultation notes. Outpatient / ambulatory referrals only.

PRIVACY

This form contains personal health information. Complete it on your own device or by hand, then print and fax to 416-864-5139. Please do not send it by email. Confirm the fax number before sending.